



Tests you can trust

Name : Shivek Sharma (22Y/M)

Date : 25 Jun 2025

Test Asked : Aarogyam C Pro Including Crm With Utsh

Report Status: Complete Report



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Controlled
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CAP From 2007

PROCESSED AT :**Thyrocare**

D-79, 3rd floor, sector-63,
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Noida, UP-201301.



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NAME : SHIVEK SHARMA (22Y/M)
REF. BY : SELF
TEST ASKED : AAROGYAM C PRO INCLUDING CRM WITH UTSH

HOME COLLECTION :
FROM Nandini Metro Suites, Sector 4, Vaishali,
Ghaziabad, Uttar Pradesh 201010 - 201010

Report Availability Summary

Note: Please refer to the table below for status of your tests.

✅ 14 Ready

🟡 0 Ready with Cancellation

🔄 0 Processing

❌ 0 Cancelled in Lab

TEST DETAILS

REPORT STATUS

AAROGYAM C PRO INCLUDING CRM WITH UTSH

Ready ✅

CHLORIDE

Ready ✅

HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)

Ready ✅

Lipoprotein (a) [Lp(a)]

Ready ✅

SODIUM

Ready ✅

TESTOSTERONE

Ready ✅

HBA PROFILE

Ready ✅

HEMOGRAM - 6 PART (DIFF)

Ready ✅

LIVER FUNCTION TESTS

Ready ✅

IRON DEFICIENCY PROFILE

Ready ✅

KIDPRO

Ready ✅

LIPID PROFILE

Ready ✅

T3-T4-USTSH

Ready ✅

VITAMIN D TOTAL AND B12 COMBO

Ready ✅

APOLIPROTEIN RATIO

Ready ✅

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NAME : SHIVEK SHARMA (22Y/M)

REF. BY : SELF

TEST ASKED : AAROGRAM C PRO INCLUDING CRM WITH UTSH

HOME COLLECTION :

FROM Nandini Metro Suites, Sector 4, Vaishali,
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Summary Report**Tests outside reference range**

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
CARDIAC RISK MARKERS			
LIPOPROTEIN (A) [LP(A)]	31.6	mg/dL	< 30
COMPLETE HEMOGRAM			
HEMATOCRIT(PCV)	38.6	%	40.0-50.0
HEMOGLOBIN	12.2	g/dL	13.0-17.0
LYMPHOCYTE	47.2	%	20-40
LYMPHOCYTES - ABSOLUTE COUNT	3.27	X 10 ³ / μ L	1.0-3.0
MEAN PLATELET VOLUME(MPV)	12.8	fL	6.5-12
PLATELET DISTRIBUTION WIDTH(PDW)	17.3	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	46.5	%	19.7-42.4
TOTAL RBC	4.33	X 10 ⁶ / μ L	4.5-5.5
LIPID			
LDL CHOLESTEROL - DIRECT	114	mg/dL	< 100
RENAL			
BUN / SR.CREATININE RATIO	25	Ratio	9:1-23:1
UREA / SR.CREATININE RATIO	53.5	Ratio	< 52
VITAMINS			
25-OH VITAMIN D (TOTAL)	16.5	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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NAME : SHIVEK SHARMA (22Y/M)**REF. BY** : SELF**TEST ASKED** : HBA PROFILE,HEMOGRAM**HOME COLLECTION :**

FROM Nandini Metro Suites, Sector 4, Vaishali,
Ghaziabad, Uttar Pradesh 201010 - 201010

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5	%

Bio. Ref. Interval. :**As per ADA Guidelines**

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
≥6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
≥8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	97	mg/dL
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Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 25 Jun 2025 08:00**Sample Received on (SRT)** : 25 Jun 2025 14:45**Report Released on (RRT)** : 25 Jun 2025 17:04**Sample Type** : EDTA Whole Blood**Labcode** : 2506086995/NCR01**Barcode** : DY357432

Dr.Sneha Singh MD(Path)

Dr.Priya Bhardwaj MD(Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

NAME : SHIVEK SHARMA (22Y/M)
REF. BY : SELF
TEST ASKED : HBA PROFILE,HEMOGRAM

HOME COLLECTION :
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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	12.2	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	38.6	%	40.0-50.0
Total RBC	HF & EI	4.33	X 10⁶/μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	89.1	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	28.2	pg	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.6	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	41.3	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.8	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	263.4	-	*Refer Note below
MENTZER INDEX	Calculated	20.6	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	6.93	X 10³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	46	%	40-80
Lymphocytes Percentage	Flow Cytometry	47.2	%	20-40
Monocytes Percentage	Flow Cytometry	4.2	%	2-10
Eosinophils Percentage	Flow Cytometry	1.7	%	1-6
Basophils Percentage	Flow Cytometry	0.6	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	3.19	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	3.27	X 10³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.29	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.04	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.12	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	155	X 10³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	12.8	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	17.3	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	46.5	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.2	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

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Sneha

Dr.Sneha Singh MD(Path)

Priya Bhardwaj

Dr.Priya Bhardwaj MD(Path)

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HOME COLLECTION :
FROM Nandini Metro Suites, Sector 4, Vaishali,
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TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	E.C.L.I.A	16.5	ng/mL

Bio. Ref. Interval. :

Deficiency : ≤ 20 ng/ml || Insufficiency : 21-29 ng/ml

Sufficiency : ≥ 30 ng/ml || Toxicity : > 100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.

Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.

Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%

Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1):87-98.

Method : Fully Automated Electrochemiluminescence Competitive Immunoassay

VITAMIN B-12	E.C.L.I.A	335	pg/mL
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Bio. Ref. Interval. :

Normal: 197-771 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L. Clinical Laboratory Diagnostics : Use and Assessment of Clinical Laboratory Results 1st Edition, TH Books-Verl-Ges, 1998:424-431

Method : Fully Automated Electrochemiluminescence Competitive Immunoassay

Please correlate with clinical conditions.

Sample Collected on (SCT) : 25 Jun 2025 08:00
Sample Received on (SRT) : 25 Jun 2025 14:55
Report Released on (RRT) : 25 Jun 2025 17:26
Sample Type : SERUM
Labcode : 2506088005/NCR01
Barcode : DY359208



Dr. Sneha Singh MD(Path)



Dr. Priya Bhardwaj MD(Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETR Y	107	mg/dL

Bio. Ref. Interval. :

Male : 86 - 152

Female : 94 - 162

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER

APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETR Y	78	mg/dL
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Bio. Ref. Interval. :

Male : 56 - 145

Female : 53 - 138

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY - BECKMAN COULTER

APO B / APO A1 RATIO (APO B/A1)	CALCULATED	0.7	Ratio
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Bio. Ref. Interval. :

Male : 0.40 - 1.26

Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
LIPOPROTEIN (A) [LP(A)]	IMMUNOTURBIDIMETRY	31.6	mg/dL
Bio. Ref. Interval. :-			

Adults : < 30.0 mg/dl

Clinical Significance:

Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:

Tietz NW, Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.

Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

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TEST NAME	TECHNOLOGY	VALUE	UNITS
TESTOSTERONE	E.C.L.I.A	523	ng/dL
Bio. Ref. Interval. :-			

280 - 800

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males. Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinemia, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 11.50 %, Inter assay (%CV): 5.70%; Sensitivity: 7 ng/dL.

Kit Validation Reference: Wilson JD Foster DW (Eds) Williams Textbook of Endocrinology 8th Edition WB Saunders Philadelphia Pennsylvania.

Note : The Biological Reference Range mentioned is specific to the age group and gender. Kindly correlate clinically.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Competitive Immunoassay

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Sample Type	: SERUM		
Labcode	: 2506088005/NCR01	Dr.Sneha Singh MD(Path)	Dr.Priya Bhardwaj MD(Path)
Barcode	: DY359208		

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FROM Nandini Metro Suites, Sector 4, Vaishali,
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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	0.58	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection, active arthritis or concurrent illness.

Clinical significance:

High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:

1. Clinical management of laboratory data in medical practice 2003-2004, 207(2003).
2. Tietz : Textbook of Clinical Chemistry and Molecular diagnostics : Second edition : Chapter 47: Page no. 1507- 1508.

Please correlate with clinical conditions.**Method:-** FULLY AUTOMATED LATEX AGGLUTINATION - BECKMAN COULTER

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FROM Nandini Metro Suites, Sector 4, Vaishali,
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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	89.3	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	370	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	24.14	%
UNSAT. IRON-BINDING CAPACITY (UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	280.7	µg/dL

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	168	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	47	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	114	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	67	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	3.5	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	1.43	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	2.4	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.41	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	120.54	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	13.5	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase

HCHO - Direct Enzymatic Colorimetric

LDL - Direct Measure

TRIG - Enzymatic, End Point

TC/H - Derived from serum Cholesterol and Hdl values

TRI/H - Derived from TRIG and HDL Values

LDL/ - Derived from serum HDL and LDL Values

HD/LD - Derived from HDL and LDL values.

NHDL - Derived from serum Cholesterol and HDL values

VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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HOME COLLECTION :

FROM Nandini Metro Suites, Sector 4, Vaishali,
 Ghaziabad, Uttar Pradesh 201010 - 201010

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	85.7	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.66	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.12	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.54	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	13.04	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	19.39	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	12.2	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	1.59	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.8	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.54	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.26	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.39	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method

BILT - Vanadate Oxidation

BILD - Vanadate Oxidation

BILI - Derived from serum Total and Direct Bilirubin values

GGT - Modified IFCC method

SGOT - IFCC* Without Pyridoxal Phosphate Activation

SGPT - IFCC* Without Pyridoxal Phosphate Activation

OT/PT - Derived from SGOT and SGPT values.

PROT - Biuret Method

SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)

SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES

A/GR - Derived from serum Albumin and Protein values

Sample Collected on (SCT) : 25 Jun 2025 08:00

Sample Received on (SRT) : 25 Jun 2025 14:55

Report Released on (RRT) : 25 Jun 2025 17:26

Sample Type : SERUM

Labcode : 2506088005/NCR01

Barcode : DY359208

Dr.Sneha Singh MD(Path)

Dr.Priya Bhardwaj MD(Path)

PROCESSED AT :**Thyrocare**D-79, 3rd floor, sector-63,
gautam budh nagar,
Noida, UP-201301.

Tests you can trust

Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 9870666333 wellness@thyrocare.com

First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]**NAME** : SHIVEK SHARMA (22Y/M)
REF. BY : SELF
TEST ASKED : AAROGYAM C PRO INCLUDING CRM WITH UTSH**HOME COLLECTION :**
FROM Nandini Metro Suites, Sector 4, Vaishali,
Ghaziabad, Uttar Pradesh 201010 - 201010

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	19.5	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.78	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	25	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	41.73	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	53.5	Ratio	< 52
CALCIUM	PHOTOMETRY	9.7	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	6.33	mg/dL	4.2 - 7.3
SODIUM	I.S.E - INDIRECT	141.1	mmol/L	136 - 145
CHLORIDE	I.S.E - INDIRECT	106.1	mmol/L	98 - 107

Please correlate with clinical conditions.**Method :**BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method
SOD - ION SELECTIVE ELECTRODE - INDIRECT
CHL - ION SELECTIVE ELECTRODE - INDIRECT**Sample Collected on (SCT)** : 25 Jun 2025 08:00
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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

NAME : SHIVEK SHARMA (22Y/M)
REF. BY : SELF
TEST ASKED : AAROGYAM C PRO INCLUDING CRM WITH UTSH

HOME COLLECTION :
FROM Nandini Metro Suites, Sector 4, Vaishali,
Ghaziabad, Uttar Pradesh 201010 - 201010

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	107	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	8.95	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	3.75	µIU/mL	0.54-5.30

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Competitive Immunoassay

USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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Dr.Sneha Singh MD(Path)

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PROCESSED AT :**Thyrocare**

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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

NAME : SHIVEK SHARMA (22Y/M)
REF. BY : SELF
TEST ASKED : AAROGYAM C PRO INCLUDING CRM WITH UTSH

HOME COLLECTION :
FROM Nandini Metro Suites, Sector 4, Vaishali,
Ghaziabad, Uttar Pradesh 201010 - 201010

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	129	mL/min/1.73 m ²
Bio. Ref. Interval. :-			

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

Sample Collected on (SCT) : 25 Jun 2025 08:00
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Sample Type : SERUM
Labcode : 2506088005/NCR01
Barcode : DY359208



Dr. Sneha Singh MD(Path)

Dr. Priya Bhardwaj MD(Path)

Page : 13 of 15

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

CUSTOMER DETAILS

As declared in our data base

Name: SHIVEK SHARMA **Age:** 22Y **Sex:** M

Barcodes/Sample_Type : DY357432 (EDTA),DY359208 (SERUM)
Labcode : 2506086995,2506088005
Ref By : SELF
Sample_Type/Tests : EDTA:HBA PROFILE , HEMOGRAM - 6 PART (DIFF)
SERUM:AAROGYAM C PRO INCLUDING CRM WITH UTSH
Sample Collected At : FROM Nandini Metro Suites,Sector 4, Vaishali, Ghaziabad, Uttar Pradesh
201010 - 201010
Sample Collected on (SCT) : 25 Jun 2025 08:00
Report Released on (RRT) : 25 Jun 2025 17:26
Amount Collected : -

Thyrocare,D-37/1,MIDC,Turbhe,Navi Mumbai - 400703. | Phone:022 - 6712 3400 | www.thyrocare.com | info@thyrocare.com

CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- ✓ Thyrocare Discovery video link :- <https://youtu.be/nbdYeRgYyQc>

EXPLANATIONS

- ✓ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ✓ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ✓ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ Testing or retesting should be done in accredited laboratories.
- ✓ For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**

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*T&C Apply, #As on 5th December 2024, *As per a survey on doctors' perception of laboratory diagnostics (IJARIIT,2023)